

Division of Health Service Regulation

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: MHL001-149	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING _____	(X3) DATE SURVEY COMPLETED R 09/05/2025
NAME OF PROVIDER OR SUPPLIER JUST IN TIME YOUTH SERVICES		STREET ADDRESS, CITY, STATE, ZIP CODE 1710 SYKES STREET BURLINGTON, NC 27215		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
V 000	INITIAL COMMENTS An annual, complaint and follow up survey was completed on September 5, 2025. The complaint was unsubstantiated (intake #NC00232848). Deficiencies were cited. This facility is licensed for the following service category: 10A NCAC 27G .1700 Residential Treatment Staff Secure for Children or Adolescents. This facility is licensed for 4 and has a current census of 2. The survey sample consisted of audits of 2 current clients and 1 former client.	V 000		
V 132	G.S. 131E-256(G) HCPR-Notification, Allegations, & Protection G.S. §131E-256 HEALTH CARE PERSONNEL REGISTRY (g) Health care facilities shall ensure that the Department is notified of all allegations against health care personnel, including injuries of unknown source, which appear to be related to any act listed in subdivision (a)(1) of this section. (which includes: a. Neglect or abuse of a resident in a healthcare facility or a person to whom home care services as defined by G.S. 131E-136 or hospice services as defined by G.S. 131E-201 are being provided. b. Misappropriation of the property of a resident in a health care facility, as defined in subsection (b) of this section including places where home care services as defined by G.S. 131E-136 or hospice services as defined by G.S. 131E-201 are being provided. c. Misappropriation of the property of a healthcare facility. d. Diversion of drugs belonging to a health care facility or to a patient or client.	V 132		

Division of Health Service Regulation

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

Division of Health Service Regulation

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V 132	Continued From page 1 e. Fraud against a health care facility or against a patient or client for whom the employee is providing services). Facilities must have evidence that all alleged acts are investigated and must make every effort to protect residents from harm while the investigation is in progress. The results of all investigations must be reported to the Department within five working days of the initial notification to the Department. This Rule is not met as evidenced by: Based on record review and interviews, the facility failed to ensure an allegation of abuse was investigated and the results of investigation were reported to Health Care Personnel Registry (HCPR) within five working days of the initial notification. The findings are: Review on 8/19/25 of the North Carolina (NC) Incident Response Improvement System (IRIS) revealed: -There was no Level III incident report submitted by the facility for an allegation of abuse against the Program Director (PD). It was alleged the PD assaulted Former Client (FC) #3. -The 5-day working report was not submitted by facility staff. Review on 8/19/25 of facility records revealed: -There was no documentation to indicate the Associate Professional (AP)/Licensee conducted an investigation for the issue above. Interview on 8/20/25 with the local Department of Social Services (DSS) Investigator revealed: -A law enforcement officer reported the incident to her related to the PD and FC #3. -They investigated the incident with the PD and FC #3.	V 132		

Division of Health Service Regulation

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V 132	Continued From page 2 -The allegation of abuse against the PD and FC #3 was later unsubstantiated by DSS. Interview on 8/20/25 with the AP/Licensee revealed: -The PD brought the allegation to her attention because a DSS Investigator showed up to the facility. -"It was said by a police officer that [PD] punched [FC #3] in his face in July 2025." -"I did an investigation, however I don't always document the investigations." -"I did not document anything related to this incident." -She confirmed the facility failed to ensure an allegation of abuse was investigated and the results of investigation were reported to HCPR within five working days of the initial notification. This deficiency constitutes a re-cited deficiency and must be corrected within 30 days.	V 132		
V 366	27G .0603 Incident Response Requirements 10A NCAC 27G .0603 INCIDENT RESPONSE REQUIREMENTS FOR CATEGORY A AND B PROVIDERS (a) Category A and B providers shall develop and implement written policies governing their response to level I, II or III incidents. The policies shall require the provider to respond by: (1) attending to the health and safety needs of individuals involved in the incident; (2) determining the cause of the incident; (3) developing and implementing corrective measures according to provider specified timeframes not to exceed 45 days; (4) developing and implementing measures to prevent similar incidents according to provider	V 366		

Division of Health Service Regulation

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V 366	Continued From page 3 specified timeframes not to exceed 45 days; (5) assigning person(s) to be responsible for implementation of the corrections and preventive measures; (6) adhering to confidentiality requirements set forth in G.S. 75, Article 2A, 10A NCAC 26B, 42 CFR Parts 2 and 3 and 45 CFR Parts 160 and 164; and (7) maintaining documentation regarding Subparagraphs (a)(1) through (a)(6) of this Rule. (b) In addition to the requirements set forth in Paragraph (a) of this Rule, ICF/MR providers shall address incidents as required by the federal regulations in 42 CFR Part 483 Subpart I. (c) In addition to the requirements set forth in Paragraph (a) of this Rule, Category A and B providers, excluding ICF/MR providers, shall develop and implement written policies governing their response to a level III incident that occurs while the provider is delivering a billable service or while the client is on the provider's premises. The policies shall require the provider to respond by: (1) immediately securing the client record by: (A) obtaining the client record; (B) making a photocopy; (C) certifying the copy's completeness; and (D) transferring the copy to an internal review team; (2) convening a meeting of an internal review team within 24 hours of the incident. The internal review team shall consist of individuals who were not involved in the incident and who were not responsible for the client's direct care or with direct professional oversight of the client's services at the time of the incident. The internal review team shall complete all of the activities as follows:	V 366		

Division of Health Service Regulation

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V 366	Continued From page 4 (A) review the copy of the client record to determine the facts and causes of the incident and make recommendations for minimizing the occurrence of future incidents; (B) gather other information needed; (C) issue written preliminary findings of fact within five working days of the incident. The preliminary findings of fact shall be sent to the LME in whose catchment area the provider is located and to the LME where the client resides, if different; and (D) issue a final written report signed by the owner within three months of the incident. The final report shall be sent to the LME in whose catchment area the provider is located and to the LME where the client resides, if different. The final written report shall address the issues identified by the internal review team, shall include all public documents pertinent to the incident, and shall make recommendations for minimizing the occurrence of future incidents. If all documents needed for the report are not available within three months of the incident, the LME may give the provider an extension of up to three months to submit the final report; and (3) immediately notifying the following: (A) the LME responsible for the catchment area where the services are provided pursuant to Rule .0604; (B) the LME where the client resides, if different; (C) the provider agency with responsibility for maintaining and updating the client's treatment plan, if different from the reporting provider; (D) the Department; (E) the client's legal guardian, as applicable; and (F) any other authorities required by law.	V 366		

Division of Health Service Regulation

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V 366	Continued From page 5 This Rule is not met as evidenced by: Based on record review and interview, the facility failed to implement a policy governing their response to Level III incidents as required. The findings are: Reviews on 8/19/25 and 8/21/25 of Former Client (FC) #3's record revealed: -Admission date of 5/24/24. -Diagnoses of Disruptive Mood Dysregulation Disorder, Adjustment Disorder with Mixed Disturbance of Emotions and Conduct and Attention Deficit Hyperactivity Disorder. -Discharge date of 7/15/25. -He was 12 years old. Review on 8/19/25 of the North Carolina (NC) Incident Response Improvement System (IRIS) revealed: -There was no level III incident report submitted by the facility for an allegation of abuse against the Program Director (PD). It was alleged the PD assaulted FC #3. -There was no documentation to determine: The cause of the incident; If the facility developed and implemented corrective measures according to the provider specified timeframes not to exceed 45 days; no measures to prevent similar incidents according to provider specified timeframes not to exceed 45 days and assigning person(s) to be responsible for implementation of the corrections and preventive measures.	V 366			

Division of Health Service Regulation

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V 366	Continued From page 6 Interview on 8/20/25 with the Associate Professional/Licensee revealed: -The PD brought the allegation to her attention because a Department of Social Services Investigator showed up to the facility. -"It was said by a police officer that [PD] punched [FC #3] in his face in July 2025." -"[The PD] did the incident reports for most of the incidents." -She confirmed the facility failed to implement a policy governing their response to Level III incidents as required.	V 366		
V 367	27G .0604 Incident Reporting Requirements 10A NCAC 27G .0604 INCIDENT REPORTING REQUIREMENTS FOR CATEGORY A AND B PROVIDERS (a) Category A and B providers shall report all level II incidents, except deaths, that occur during the provision of billable services or while the consumer is on the providers premises or level III incidents and level II deaths involving the clients to whom the provider rendered any service within 90 days prior to the incident to the LME responsible for the catchment area where services are provided within 72 hours of becoming aware of the incident. The report shall be submitted on a form provided by the Secretary. The report may be submitted via mail, in person, facsimile or encrypted electronic means. The report shall include the following information: (1) reporting provider contact and identification information; (2) client identification information; (3) type of incident; (4) description of incident; (5) status of the effort to determine the	V 367		

Division of Health Service Regulation

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V 367	Continued From page 7 cause of the incident; and (6) other individuals or authorities notified or responding. (b) Category A and B providers shall explain any missing or incomplete information. The provider shall submit an updated report to all required report recipients by the end of the next business day whenever: (1) the provider has reason to believe that information provided in the report may be erroneous, misleading or otherwise unreliable; or (2) the provider obtains information required on the incident form that was previously unavailable. (c) Category A and B providers shall submit, upon request by the LME, other information obtained regarding the incident, including: (1) hospital records including confidential information; (2) reports by other authorities; and (3) the provider's response to the incident. (d) Category A and B providers shall send a copy of all level III incident reports to the Division of Mental Health, Developmental Disabilities and Substance Abuse Services within 72 hours of becoming aware of the incident. Category A providers shall send a copy of all level III incidents involving a client death to the Division of Health Service Regulation within 72 hours of becoming aware of the incident. In cases of client death within seven days of use of seclusion or restraint, the provider shall report the death immediately, as required by 10A NCAC 26C .0300 and 10A NCAC 27E .0104(e)(18). (e) Category A and B providers shall send a report quarterly to the LME responsible for the catchment area where services are provided. The report shall be submitted on a form provided by the Secretary via electronic means and shall	V 367		

Division of Health Service Regulation

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V 367	Continued From page 8 include summary information as follows: (1) medication errors that do not meet the definition of a level II or level III incident; (2) restrictive interventions that do not meet the definition of a level II or level III incident; (3) searches of a client or his living area; (4) seizures of client property or property in the possession of a client; (5) the total number of level II and level III incidents that occurred; and (6) a statement indicating that there have been no reportable incidents whenever no incidents have occurred during the quarter that meet any of the criteria as set forth in Paragraphs (a) and (d) of this Rule and Subparagraphs (1) through (4) of this Paragraph. This Rule is not met as evidenced by: Based on record review and interview, the facility failed to ensure an incident was reported to the Local Management Entity/Managed Care Organization (LME/MCO) for the catchment area where services are provided within 72 hours of becoming aware of the incident. The findings are: Reviews on 8/19/25 and 8/21/25 of Former Client (FC) #3's record revealed: -Admission date of 5/24/24. -Diagnoses of Disruptive Mood Dysregulation Disorder, Adjustment Disorder with Mixed Disturbance of Emotions and Conduct and Attention Deficit Hyperactivity Disorder. -Discharge date of 7/15/25.	V 367		

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V 367	Continued From page 9 -He was 12 years old. Review on 8/19/25 of the North Carolina (NC) Incident Response Improvement System (IRIS) revealed: -There was no level III incident report submitted by the facility for an allegation of abuse against the Program Director (PD). It was alleged the PD assaulted FC #3. Interview on 8/20/25 with the Associate Professional/Licensee revealed: -The PD brought the allegation to her attention because a Department of Social Services Investigator showed up to the facility. -"It was said by a police officer that [PD] punched [FC #3] in his face in July 2025." -"[The PD] did the incident reports for most of the incidents." -She confirmed the facility failed to report the above incident to LME/MCO within 72 hours. This deficiency constitutes a re-cited deficiency and must be corrected within 30 days.	V 367		
V 736	27G .0303(c) Facility and Grounds Maintenance 10A NCAC 27G .0303 LOCATION AND EXTERIOR REQUIREMENTS (c) Each facility and its grounds shall be maintained in a safe, clean, attractive and orderly manner and shall be kept free from offensive odor. This Rule is not met as evidenced by: Based on observation and interview, the facility was not maintained in a clean, attractive and orderly manner. The findings are:	V 736		

Division of Health Service Regulation

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V 736	Continued From page 10 Observation on 8/21/25 at approximately 11:35 am revealed: -Client #2's bedroom-Paint peeling on walls. Black stains on wall near bed. Wooden floor had approximately 50 black scrapes. -Empty bedroom-Approximately 25 black and grays scrapes on wooden floor. -Bathroom in hallway-Blind slats were crumbled and 3 slats were missing. Porcelain toilet paper holder was cracked. Peeling paint on walls. Linoleum flooring had pink paint and grayish stains. -Client #1's bedroom-One of the glass panes was missing from the window. The bigger window's inside glass pane was missing. The curtain rods were bent, and the curtains were sagging. Interview on 8/21/25 with the Program Director revealed: - He was not sure what happened to the windows in client #1's bedroom. -He was aware of most of the maintenance issues with the facility. -He acknowledged all of the above issues with the facility.	V 736		